

SBAR Hand-Off Pilot Source Packet

Digital portion — 7 of 10 items

YOUR TASK

You are LTC Pat Donovan, Deputy Commander for Administration at Stewart Army Community Hospital (100 beds). The Chief of Staff has tasked you with writing an MFR documenting the 90-day SBAR hand-off pilot on Med-Surg and ICU, and recommending whether to expand it enterprise-wide. Source material follows. There is more here than fits in a one-page MFR. **Curate, write, sign.**

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1. Tasking memo from the Chief of Staff
2. Voice memo transcript — Chief Nursing Officer
3. Pre-pilot baseline data (Q1)
4. 90-day pilot data (Apr–Jun)
5. Reference MFR — similar protocol, different hospital
6. Email thread — two physicians
7. Chief Resident's overnight phone notes

Three additional source items (scribbled huddle notes, pilot scorecard, family thank-you letter) are on the side table at the front of the room. Two copies of each. First come, first served.

MEMORANDUM FOR LTC Patrick J. Donovan, Deputy Commander for Administration

SUBJECT: MFR – SBAR Hand-Off Pilot, Recommendation for Enterprise Implementation

Pat –

Give me an MFR documenting the 90-day SBAR pilot on Med-Surg and ICU. Cover what we did, what the data showed, what the staff said, and your recommendation on enterprise rollout.

I want to mention the Whitfield catch (Med-Surg, Bed 412, 14 FEB) in the brief – daughter sent a letter, see attached. Use it or don't, your call.

Need it for the Commander's brief Friday. One page if you can manage it.

– COL Marcus T. Reeves, Chief of Staff

(912) 767-XXXX | marcus.t.reeves2.mil@health.mil

Speaker: LTC Diane Park, Chief Nursing Officer

Recorded: 0612, driving in to work

Duration: 2:47 | **Transcription:** SuperWhisper, light hand-edit

“OK so, my honest read on the SBAR pilot. Mixed bag. The Med-Surg charge nurses are mostly on board. Williams and Choi both came up to me last week unprompted and said the structured format actually saved them on a couple of complex transfers — they would have missed the Coumadin schedule on Mrs. Whitfield in 412 without the template. I think that's real.”

“ICU is more skeptical. Walking around at change of shift it adds maybe two, three minutes per patient compared to the old verbal hand-off. When you've got eight patients and you're trying to get out the door for daycare pickup that adds up. Master Sergeant Henderson — Donna, you know how she is — she basically said we've taken something that worked fine and made it bureaucratic. She's not wrong that it's slower. She is wrong that the old way worked fine, in my opinion.”

“The data is what it is. We had — I think it was four — preventable hand-off-related events in the prior quarter, zero this quarter on the pilot units. Sample size is small, ninety days, two units, but it tracks with what the literature says. The thing I'd flag for you is the near-miss reports went up. Some people will read that as “errors went up” but I think we're just catching things we used to miss. The Castellanos case in ICU Bed 7 — the K+ trend the chief resident caught — would not have been caught under the old hand-off, period.”

“Patient feedback — there's that letter from Margaret Whitfield-Pearson, Eleanor Whitfield's daughter, the one who said the night nurse caught the wrong Coumadin dose because she went through the SBAR with the day nurse and noticed the discrepancy. That's the kind of thing that's hard to put a number on but is exactly why we did this.”

“Anyway. My vote is expand it but trim the template. The current version is too long for stable patients. Make it scaled — intensive for complex patients, lighter for routine. Call me if you want to talk it through. Done.”

Pre-Pilot Baseline (Q1)

Med-Surg and ICU · January–March

Metric	Jan	Feb	Mar	Q1 Avg
Hand-off-related safety events	2	1	1	1.3 / mo
Avg hand-off duration (min/patient)	1.8	2.1	1.9	1.9
Near-miss reports (hand-off related)	3	4	2	3.0 / mo
Staff satisfaction (1–5, monthly survey)	3.6	3.5	3.7	3.6
Patient/family communication complaints	5	7	4	5.3 / mo

Source: PSO Quality Council monthly export. Pilot units only (Med-Surg + ICU, n=42 beds combined).

90-Day Pilot (Apr–Jun)

Same units, same metrics · Δ vs. Q1 baseline

Metric	M1 (Apr)	M2 (May)	M3 (Jun)	Avg	Δ vs. Baseline
Hand-off-related safety events	1	0	0	0.3 / mo	-77%
Avg hand-off duration (min/patient)	4.2	3.8	3.5	3.8	+100%
Near-miss reports (hand-off related)	6	8	7	7.0 / mo	+133%
Staff satisfaction (1–5, monthly survey)	3.1	3.3	3.6	3.3	-0.3
Patient/family communication complaints	4	3	2	3.0 / mo	-43%

Source: PSO Quality Council monthly export. PSO note: near-miss increase assessed as improved detection, not degraded performance.

DEPARTMENT OF THE ARMY
CARL R. DARNALL ARMY MEDICAL CENTER
36000 DARNALL LOOP
FORT HOOD, TX 76544

MCXX-DCA

15 March 2023

MEMORANDUM FOR RECORD

SUBJECT: Implementation Outcomes – Standardized Hand-Off
Communication Protocol, Inpatient Wards (Pilot 1 OCT 2022
– 31 DEC 2022)

1. References.

- a. AR 40-66, Medical Record Administration.
- b. The Joint Commission, National Patient Safety Goal 2.
- c. AHRQ TeamSTEPPS Pocket Guide, 3.0.

2. Purpose. To document the outcomes of the 90-day pilot of the I-PASS hand-off communication protocol on three inpatient wards, and to recommend enterprise expansion.

3. Background. Hand-off communication failures contributed to 11 of 23 sentinel events reviewed by the Patient Safety Committee in FY22. The pilot tested I-PASS, a five-element structured hand-off, on Med-Surg, Step-Down, and Mother-Baby units between 1 OCT and 31 DEC 2022.

4. Findings.

- a. Hand-off-related safety events on pilot units decreased from seven to two over the 90-day period (71 percent reduction).
- b. Mean hand-off duration increased from 2.1 to 3.4 minutes per patient. Staff initially expressed frustration with the added time; satisfaction scores recovered to within 0.2 points of baseline by month three.
- c. Near-miss reporting on pilot units increased 60 percent. The Patient Safety Committee judged this attributable to improved detection rather than degraded performance.
- d. Patient and family communication complaints decreased 38 percent on pilot units while remaining flat hospital-wide.

5. Recommendation. Expand I-PASS to all inpatient units beginning 1 APR 2023. Implementation should include a 30-day ramp per ward with dedicated nurse-educator support.

6. Point of contact is the undersigned at (254) 555-XXXX or margaret.l.huynh.mil@health.mil.

MARGARET L. HUYNH
LTC, MS
Deputy Commander for
Administration

FROM MAJ Daniel J. Forrester, MD <daniel.j.forrester.mil@health.mil>
TO MAJ Sarah J. Chen, MD <sarah.j.chen.mil@health.mil>
SENT Tuesday 10:47 AM

SBAR is killing rounds

Sarah —

I've timed it three days running this week. Old hand-off: ~12 min for the whole list. New SBAR hand-off: 26 min. Twenty-six. I'm losing ~14 minutes every morning before rounds even start.

I'm not anti-protocol but the template is overbuilt for stable patients. We do not need a five-element formal hand-off on the post-op gallbladder who's eating breakfast and going home (Garrison, 416, ate a full breakfast this morning and is going home before noon). Can we push back on this with Park before they expand it?

— Dan

MAJ Daniel J. Forrester, MD
General Surgery, Stewart Army Community Hospital
DOD ID: 1234567890 | (912) 767-XXXX

FROM MAJ Sarah J. Chen, MD <sarah.j.chen.mil@health.mil>
TO MAJ Daniel J. Forrester, MD <daniel.j.forrester.mil@health.mil>
SENT Tuesday 1:51 PM

RE: SBAR is killing rounds

Dan —

Time is real and I hear you. But I caught a med reconciliation error on Tuesday that I would have missed under the old hand-off — Mrs. Whitfield, 412, on warfarin, INR drawn night shift, day team didn't know it had been resulted. The structured format flagged it.

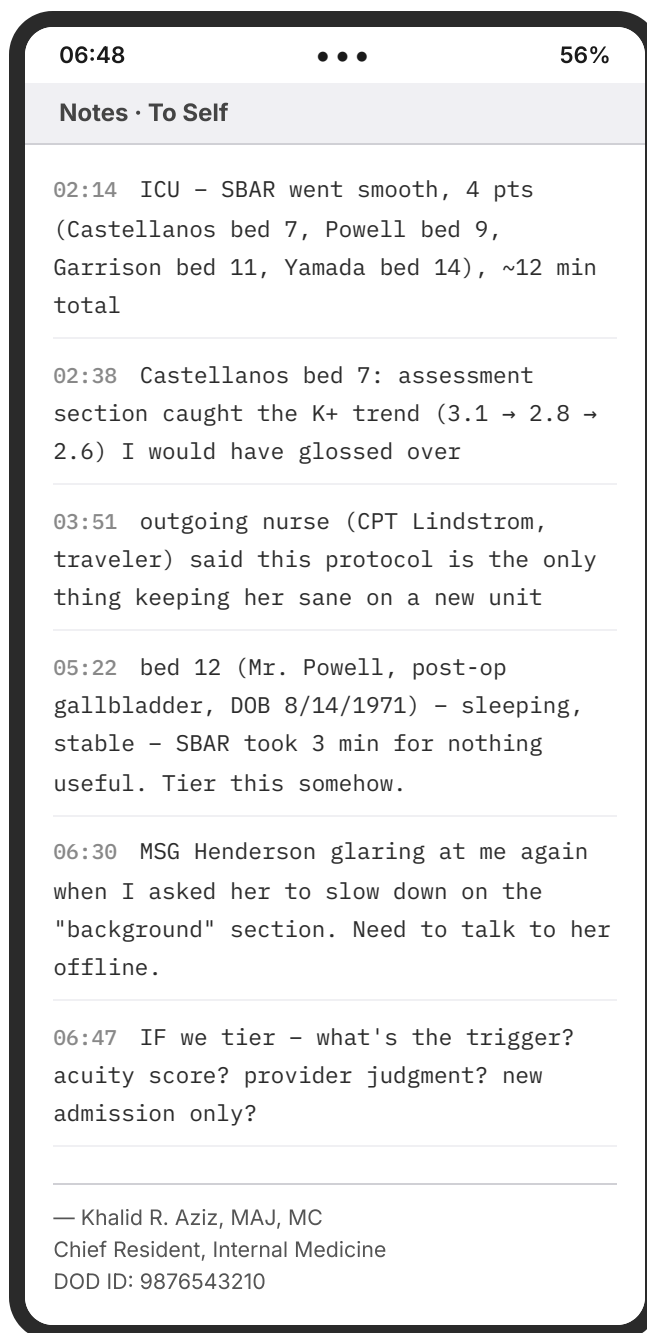
I think the answer is "trim the template for stable patients" not "kill it." Want to draft something jointly to Park instead of pushing back?

— Sarah

FROM MAJ Daniel J. Forrester, MD <daniel.j.forrester.mil@health.mil>
TO MAJ Sarah J. Chen, MD <sarah.j.chen.mil@health.mil>
SENT Tuesday 4:33 PM

RE: RE: SBAR is killing rounds

Fine. Tier the template. Long form for ICU and complex Med-Surg, short form for the rest. I'll draft something this weekend.



Forwarded to LTC Donovan via screenshot, with permission, by MAJ Aziz.